TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 14:35
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Complete Blood Count (Hemogram)

Test		Results	Unit	Bio. Ref. Interval
Hemoglobin (Colorimetric)		13.3		12.0 - 15.0
RBC Count (Impedance)		4.69	X 10^12/L	3.8 - 4.8
Hematrocrit (Calculated)		45.5	%	36 - 46
MCV (RBC Histogram)		97.1	fL	83 - 101
MCH (calculated)		28.3	pg	27 - 32
MCHC (calculated)	L	**29.2**	g/dL	31.5 - 34.5
RDW (RBC Histogram)		13.8	%	11.5 - 14.5
NRBC(Flowcytometry)		0	/100 WBCs	0 - 0
WBC Count (Impedance & flow)	H	**10120**	/cmm	4000 - 10000

DIFFERENTIAL WBC COUNT
(Flowcytometry and Microscopic Examination)

	[%]		EXPECTED VALUES	[Abs]		EXPECTED VALUES
Neutrophils	64.1	%	30 - 75	6490	/cmm	1800 - 7700
Lymphocytes	28.0	%	21 - 49	2830	/cmm	1000 - 3900
Monocytes	5.1	%	3 - 11	520	/cmm	200 - 800
Eosinophils	2.3	%	0 - 7	230	/cmm	20 - 500
Basophils	0.5	%	0 - 2	50	/cmm	0 - 100
Immature Granulocytes	0.2	%	0 - 0.5			

Platelet Count (Impedance or Optical)		280000	/cmm	150000 - 410000
MPV (Platelet Histogram)	H	**12.2**	fL	7.2 - 11.7

Sample Type: EDTA Whole Blood

Note: All abnormal hemograms are reviewed and confirmed microscopically.Peripheral blood smear and malarial parasite examination are not part of CBC report.

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Test done from collected sample.

Approved by:

Dr. Avinash B Panchal
MBBS,DCP
G-44623

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 14:55
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Reticulocyte count (Automated)

Metho: Flowcytometry; Specimen: EDTA whole blood

Test Name	Results	Units	Bio. Ref. Interval
Reticulocyte count *Flow Cytometry*	1.79	%	0.5 - 2.0
Absolute reticulocyte count *Calculated*	84.00	X 10^9/L	50 - 100
Reticulocyte production index *Calculated*	1.90	Index	>2 : Significant increase
Immature reticulocyte fraction *Calculated*	7.5	%	2.0 - 16.5
RET- He (CHr) *Calculated*	**26.3**	pg	28 - 35

Sample Type: EDTA Whole Blood

- Reticulocyte % / absolute reticulocyte count / reticulocyte production index indicates the quantity of circulating reticulocytes.
- Immature reticulocyte fraction (IRF) is a measure of an early reticulocyte response.
- RET-He is a measure of the amount of hemoglobin in each reticulocyte, reflecting quality of the newly produced reticulocytes. If RET-He is decreased, it is an indication of inadequate iron supply to the bone marrow relative to demand. An exception would include patients with thalassemia who may have lower RET-He.
- RET-He is also usuful in monitoring response to iron as well as erythropoietin therapy. Under only succesful therapy Ret-He is expected to rise after 2 days and this is not influenced by presence or absence of inflammation.

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Test done from collected sample.

Approved by:

Dr. Mohan Galande
M.D. Pathology
G-10116

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 16:09
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Test Name	Results	Units	Bio. Ref. Interval
SERUM PROTEIN WITH A/G RATIO			
TOTAL PROTEIN *Method:Biuret*	7.80	g/dL	6.4 - 8.3
ALBUMIN *Bromo-Cresol Green*	4.65	g/dL	3.5 - 5.2
GLOBULIN *Calculated*	3.15	g/dL	2.4 - 3.5
ALB/GLB Ratio *Calculated*	1.48		1.2 - 2.2

Sample Type: Serum

CLINICAL SIGNIFICANCE:

- Changes in the relative percentage of plasma proteins can be due to a change in the percentage of one plasma protein fraction. Often in such cases the amount of total protein does not Change.
- The A/G ratio is commonly used as an index of the distribution of Albumin and globulin fractions.
- Marked changes in this ratio can be Observed in cirrhosis of the liver, glomerulonephritis, nephrotic syndrome, Acute hepatitis, lupus erythematosus as well as in certain acute and chronic Inflammations.
- Total protein measurements are used in the diagnosis and treatment of a variety of diseases involving the liver, kidney, or bone Marrow, as well as other metabolic or nutritional disorders

HYPOPROTEINEMIA:

- Loss Of Blood,
- Sprue,
- Nephrotic Syndrome,
- Severe Burns,
- Salt Retention Syndrome
- Kwashiorkor (Acute Protein Deficiency).

HYPERPROTEINEMIA:

- Severe Dehydration
- Multiple Myeloma.

This is an electronically authenticated report."Please verify the authenticity of this report by scanning the QR code to ensure data integrity."

Test done from collected sample.

Approved by:

Dr. Pallavi Jain
M.D. Biochemistry
Reg. No.:-83546

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 16:05
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

IMMUNOGLOBULIN PROFILE

Test Name	Results	Units	Bio. Ref. Interval
Immunoglobulin A (IgA) *Nephelometry*	1.67	g/L	0.70 - 4.00
Immunoglobulin M (IgM) *Nephelometry*	1.41	g/L	0.4 - 2.30
Immunoglobulin G (IgG) *Nephelometry*	14.2	g/L	7 - 16

Sample Type: Serum

An immunoglobulin profile measures the amount of immunoglobulins in your blood. Immunoglobulins are also called antibodies, which are proteins that your immune system makes to fight germs.

High immunoglobulin levels might be caused by:

- Allergies
- Chronic infections
- An autoimmune disorder that makes your immune system overreact, such as rheumatoid arthritis, lupus, or celiac disease
- Liver disease

Symptoms of a deficiency in IgG, IgA, or IgM include:

- Frequent or severe infections, such as sinusitis and pneumonia

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Test done from collected sample.

Approved by:

Dr. Rina Prajapati
D.C.P. DNB (Path)
G-21793

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 16:10
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Test Name	Results	Units	Bio. Ref. Interval
Creatinine *Enzymatic*	H **1.43**	mg/dL	0.55 - 1.02

Creatinine is the most common test to assess kidney function. Creatinine levels are converted to reflect kidney function by factoring in age and gender to produce the eGFR (estimated Glomerular Filtration Rate). As the kidney function diminishes, the creatinine level increases; the eGFR will decrease. Creatinine is formed from the metabolism of creatine and phosphocreatine, both of which are principally found in muscle. Thus the amount of creatinine produced is, in large part, dependent upon the individual's muscle mass and tends not to fluctuate much from day-to-day. Creatinine is not protein bound and is freely filtered by glomeruli. All of the filtered creatinine is excreted in the urine.

Test Name	Results	Units	Bio. Ref. Interval
Uric Acid (UA) *Uricase*	H **6.38**	mg/dL	2.5 - 6.2

Most uric acid is synthesized in the liver and mainly excreted by kidney. It is an end product of purine catabolism. Levels are labile and show day to day and seasonal variation in same person. Levels are also increased by emotional stress, total fasting and increased body weight. Mainly used for monitoring treatment of gout and chemotherapeutic treatment of neoplasm. Levels are increased in renal failure, gout, certain neoplastic condition (Increased cell turn over), Hemolytic anemia, toxemia of pregnancy.

Test Name	Results	Units	Bio. Ref. Interval
Calcium *Arsenazo III*	H **10.20**	mg/dL	8.4 - 10.0

NOTE:

Useful for- Diagnosis and monitoring of a wide range of disorders including diseases of bone, kidney, parathyroid gland or gastrointestinal tract.

Decreased calcium (hypocalcemia)

- Absence or impaired function of parathyroid gland.
- Impaired vitamin d synthesis
- Chronic renal failure
- Hypoalbuminemia

Increased calcium (hypercalcemia)

- Primary hyperparathyroidism
- Bone metastatsis of carcinomas

Test Name	Results	Units	Bio. Ref. Interval
Lactate Dehydrogenase (LDH) *Lactate To Pyruvate Uv-IFCC*	H **271.00**	U/L	125 - 220

Sample Type: Serum

Lactate dehydrogenase (LDH) activity is present in all cells of the body with the highest concentrations in the heart, liver, muscle, kidney, lung, and erythrocytes. Appearance of LDH in the serum occurs only after prolonged hypoxia and is elevated in a number of clinical conditions including cardiorespiratory diseases, malignancy, hemolysis, and disorders of the liver, kidneys, lung, and muscle. For fluid, LDH may be used to differentiate transudative from exudative effusions. Lactate dehydrogenase (LDH) activity is one of the most sensitive indicators of in vitro hemolysis. Causes can include transportation via pneumatic tube and vigorous mixing. Contamination with erythrocytes will falsely elevate results, because the analyte level in erythrocytes is higher than in normal sera.

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Test done from collected sample.

Approved by:

Dr. Parikshit Thakkar

Reg. No.:-G-15477

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 16:10
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

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Test done from collected sample.

Approved by: **Dr. Parikshit Thakkar**

Reg. No.:-G-15477

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 17:00
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Test Name		Results	Units	Bio. Ref. Interval
FREE LIGHT CHAIN ASSAY				
FREE KAPPA LIGHT CHAIN *Method:Turbidimetric*	H	**70.08**	mg/L	3.3 - 19.4
FREE LAMBDA LIGHT CHAIN, Serum *Method:Turbidimetric*		**58.71**	mg/L	5.71 - 26.3
KAPPA/LAMBDA RATIO		1.19366		Normal : 0.26 to 1.65 Renal Dysfunction : 0.37 to 3.1

Sample Type: Serum

Interpretation:

1. In patients with monoclonal lambda chain, ratio is < 0.26
2. In patients with monoclonal kappa chain, ratio is >1.65
3. Both elevated kappa and lambda free light chain (FLC) may occur due to polyclonal hypergammaglobulinemia or impaired renal clearance.

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Test done from collected sample.

Approved by:

Dr. Parikshit Thakkar

Reg. No.:-G-15477

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 15:59
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

Test Name	Results	Units	Bio. Ref. Interval
Beta 2 Microglobulin *CLIA*	H **4.26**	mg/L	0.6 - 2.45

Sample Type: Serum

ß2 microglobulin also known as B2M is a component of MHC class 1 molecules, present on all nucleated cells. Levels of ß2 microglobulin can be elevated in multiple myeloma and lymphoma.

In patients on long-term hemodialysis, it can aggregate into amyloid fibers that deposit in joint spaces, a disease, known as dialysis-related amyloidosis. It can be used in assessing renal function, particularly in kidney-transplant recipients and in patients suspected of having renal tubulointerstitial disease.

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Test done from collected sample.

Approved by:

Dr. Parikshit Thakkar

Reg. No.:-G-15477

Generated On : 28-May-2026 21:02

TEST REPORT

Reg. No.	: 60520300146 **Reg. Date** : 28-May-2026 14:19 **Ref.No** :	**Approved On**	: 28-May-2026 16:31
Name	: Mrs. SAVITA AGRWAL BH237627	**Collected On**	: 27-May-2026 14:19
Age	: 53 Years **Gender:** Female **Pass. No. :**	**Dispatch At**	:
Ref. By	: Dr. Shubh S Purohit (Hemet.Onco)	**Tele No.**	:
Location	: AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL		

SERUM PROTEIN ELECTROPHORESIS

Test Name	Results	Units	Bio. Ref. Interval
TOTAL PROTEIN *Method:Biuret*	7.80	g/dL	6.4 - 8.3
ALBUMIN	4.49	g/dL	4.02 - 4.76
GLOBULIN	3.31	g/dL	2.0 - 3.5
A/G RATIO	1.36		1.2 - 2.2
ALPHA 1	0.31	g/dL	0.21 - 0.35
ALPHA 2	0.80	g/dL	0.51 - 0.85
BETA 1	0.43	g/dL	0.34 - 0.52
BETA 2	0.39	g/dL	0.23 - 0.47
GAMMA	**1.38**	g/dL	0.80 - 1.35
INTERPRETATION	Monoclonal protein not detected. Normal protein electrophoresis pattern.		

Sample Type: Serum

METHOD: Capillary Electrophoresis

Remarks:

1. Serum immunofixation is required in the following conditions to differentiate monoclonal and polyclonal disorders.
2.
    - A well defined 'M' band
    - Faint band
    - Chronic inflammatory pattern (decreased albumin, increased alpha, increased gamma protein) which may mask the monoclonal band.
    - Isolated increase in any region, with otherwise normal pattern.
3. Shouldering of albumin peak along anodal or cathodal side may be seen with lipoproteins, drugs, bilirubin or radiological conrast.
4. Normal serum protein electrophoresis does not rule out the monoclonal gammopathy and should not be used to screen for the disorder.
5. Approximately 11% of patients with multiple myeloma patients have completely normal serum electrophoresis with the monoclonal protein only identified by immunofixamtion electrophoresis.
6. Approximately 8% of multiple myeloma patients have hypogammaglobulinemia without a quantifiable M-spike on protein electrophoresis bu identified by immunofixation electrophoresis.

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Test done from collected sample.

Approved by: **Dr. Brinda Amin**
M.D. Pathology
G-37096

Generated On : 28-May-2026 21:02

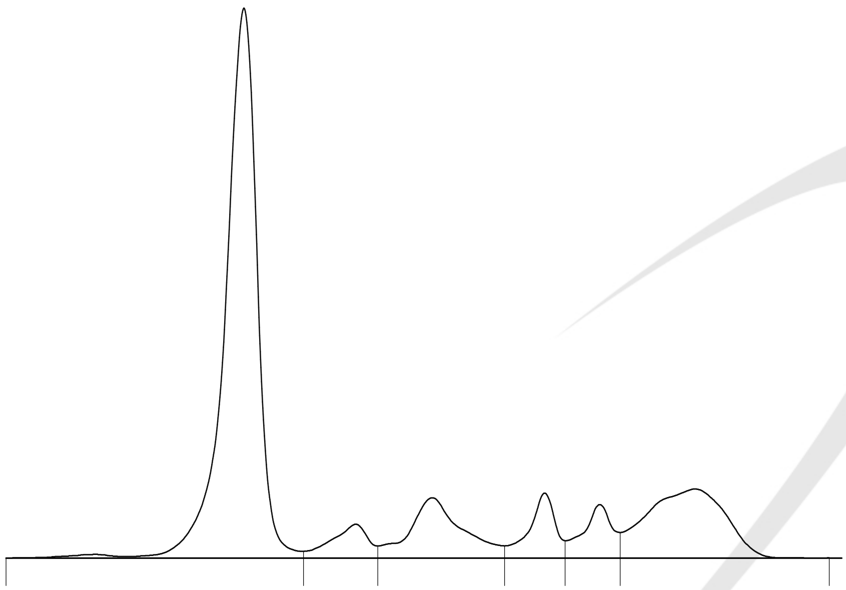
SAVITA AGRWAL BH237627

ID: **260520300146**

Sample #: **49** Date: **28/05/2026**

Age : 53

Method: Capillarys Electrophoresis by Sebia



Serum protein electrophoresis

Fractions	%	Conc. (g/dL)	Ref. (g/dL)
Albumin	**57.5**	**4.49**	4.02 - 4.76
Alpha 1	**4.0**	**0.31**	0.21 - 0.35
Alpha 2	**10.3**	**0.80**	0.51 - 0.85
Beta 1	**5.5**	**0.43**	0.34 - 0.52
Beta 2	**5.0**	**0.39**	0.23 - 0.47
Gamma	**17.7**	**1.38**>	0.80 - 1.35

A/G Ratio: **1.35**

T. P.: **7.8** g/dL

Signature

PROTEIN ELECTROPHORESIS

Interferences:

Components	**Compositions**	**Interferences**
Albumin	Albumin	Lipoproteins, drugs, bilirubin, radiological contrast.
Alpha - 1 globulins	α -1 antitrypsin, α -1 acid glycoprotein	-
Alpha - 2 globulins	α -2 macroglobulin, haptoglobulin	Haptoglobin – haemoglobin complex
Beta globulins	Transferrin, β-lipoprotein, IgA, IgM & sometimes IgG with complement protein	*Fibrinogen
Gamma globulins	IgG, IgA, IgM, IgD, IgE	CRP

***** Fibrinogen band mimic monoclonal protein in gamma fraction of protein electrophoresis. Fibrinogen is due to the use of plasma or serum not fully clot from a patient under heparin therapy or dialysis. The antisera in immunofixation will not react with fibrinogen.

Interpretation:

1. The major clinical application of serum protein electrophoresis is the detection of monoclonal immunoglobulins (paraproteins) to assist in the diagnosis and monitoring of multiple myeloma and related disorders.
2. Serum protein can be grouped in to 5 fractions by protein electrophoresis Albumin, Alpha-1, Alpha-2, Beta and Gamma globulins.
3. The concentration of these fractions and characteristic electrophoretic pattern is useful in the diagnosis of certain disorders.

Electrophoretic characteristics of serum proteins in certain clinical conditions

	Total protein	**Albumin**	**Alpha 1**	**Alpha 2**	**Beta**	**Gamma**
Acute inflammation		↓N	↑	↑		N↓
Subacute inflammation	N	N↓	N	↑	N	N
Chronic inflammation		↓N	↑	↑	N↑	↑
Sever hepatitis	↓N	↓↓	↓	↓	↓	↓
Chronic cirrhosis	↓N or ↑	↓↓		↓	↓	↑
Acute cirrhosis	↓N or ↑	↓↓		↓	Beta-gamma bridge	
Nephrotic syndrome	↓↓	↓↓		↑↑		N↓
Hypogammaglobulinemia						↓↓↓
Paraprotein	N or ↑	↓	↓	↓	Homogeneous peak	
Hyergammaglobulinemia	↑N	↓				↑
Hypoproteinemia (protein loss)	↓↓	↓↓	N↑	N↑	↓	↓N or
Alpha 1 antitrypsin deficiency			↓↓			
N: Normal, ↑: Increase, ↓: Decrease						

LABORATORY REPORT

Reg. No	:	**60520300146**	Reg. Date	:	28-May-2026 14:19
Name	:	**Mrs. SAVITA AGRWAL BH237627**	Collection on	:	27-May-2026 14:19
Sex/Age	:	**Female / 53 Years**	Report Date	:	28-May-2026 20:41
Ref. By	:	Dr. Shubh S Purohit (Hemet.Onco)	Tele. No	:	
Location	:	AYUSHMAN MEDICAL DIAGNOSTICS PVT.LTD. @ BHOPAL	Dispatch At	:	

Protein Immunofixation Electrophoresis

Specimen: Serum
Method: Agarose Gel electrophoresis
Procedure: Serum proteins are electrophoresed on Tris Barbital Buffered agarose gel and immunofixed by antisera with anti IgG, IgA, IgM heavy chains, anti-kappa and anti-lambda (free and bound) light chains. After immunofixation, the precipitated proteins are stained with acid violet.

Electrophoretic Zone	M Protein	Biological reference value
IgG	Absent	Absent
IgA	Absent	Absent
IgM	Absent	Absent
Kappa	Absent	Absent
Lambda	Absent	Absent
M protein	Absent	Absent

Impression: Monoclonal protein is not detected.

Interpretation:

Remark	M protein	Heavy chain (IgG/ IgM/IgA)	Light chain (Kappa/Lambda)	Interpretation
1	1 M protein	+	+	Presence of monoclonal
2	1 M protein	-	+	Light chain disease, suggest urine immunofixation
				IgD or IgE disease
				Multiple bands in light chain regions indicates polymerised
3	1 M protein	+	-	Heavy chain disease
4	1 faint M protein	Faint band	-	Cryoglobulin
5	2 M proteins	2 M proteins with same or different heavy chain	2 M proteins with same or different light chain	Biclonal gammopathy
				Paraprotein (monomer/polymer of immunoglobulins)

------------------ End Of Report ------------------

Dr. Brinda Amin
M.D. Pathology
G-37096

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Unipath
SPECIALTY LABORATORY Ltd.

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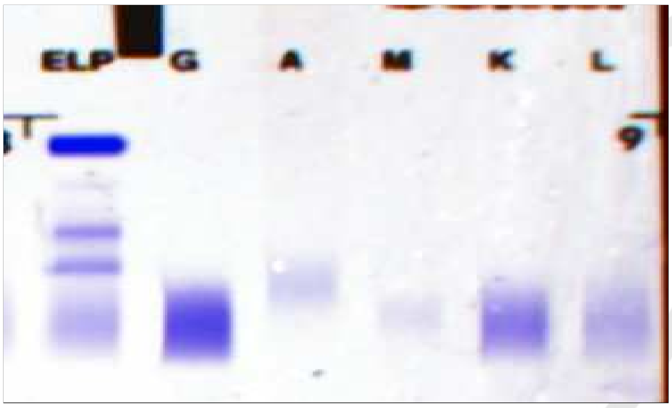
Unigenome: 2A,3A,3B PASL -House, Beside Sahjanand college, Opposite Kamdenu Complex, Panjarapole, Ambawadi, Ahmedabad-380015,Gujarat
Phone: +91 -79-49006800,07699991171 | **Whatsapp:** 6356005900 | **Email:** info@unipath.in | **Website:** www.unipath.in

SAVITA AGRWAL BH237627 Sample: **27**
ID: **260520300146** Date: **28/05/26**
Age : 53 Sex : F

Serum Immunofixation



Comment:

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